

Unlocking the House Officer Bottleneck

Letting the doctors Nigeria has already trained start work

Roughly 2,000 fully qualified doctors are stranded each year because the housemanship system has room for only 4,000 of the 6,000 who graduate. This brief sets out four connected reforms to clear that bottleneck without training a single additional doctor.

For: Prof Iziaq Adekunle Salako, Honourable Minister of State for Health and Social Welfare

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Date: July 2026

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Copied to: The Coordinating Minister of Health and Social Welfare; the Registrar, Medical and Dental Council of Nigeria

From: The Healthcare Federation of Nigeria

Executive summary

Nigeria is training more doctors than at any point in its history, and at the same time turning thousands of them away at the very first step of their careers. Roughly 6,000 doctors qualify each year, but the centralised housemanship system has room for only about 4,000. Around 2,000 fully qualified graduates are therefore left stranded every year, unable to begin the one year of supervised internship that stands between graduation and full registration. Many wait six to twelve months. Some are asked for bribes and other improper fees to secure a place.

The bribery and the frustration with the recent centralisation of placement are real, but they are symptoms. The underlying problem is structural, and it has four distinct parts: the pool of accredited training capacity is too small and too narrowly defined; graduate numbers are rising by deliberate government policy against that frozen pool; the funding of the intern post is fragile and does not follow the graduate; and the allocation mechanism is centralised, opaque, and easy to game. Left unaddressed, the gap will widen sharply, because the expansion of medical school intake now underway will push annual output well above 6,000 while internship capacity stays where it is.

The Medical and Dental Council of Nigeria has itself told the National Assembly that the answer is to bring state and private hospitals into the system. The Healthcare Federation of Nigeria agrees, and goes further. This brief sets out four connected reforms, each targeted at one of the four root causes, that together would clear the backlog, protect training quality, and turn a bottleneck into an on-ramp for the workforce Nigeria is investing so heavily to create.

1. The problem, stated plainly

A Nigerian medical graduate cannot practise, cannot be fully registered, and in most cases cannot proceed to residency or leave for national service until completing a twelve-month housemanship: three months each, uninterrupted, in Internal Medicine, Surgery, Obstetrics and Gynaecology, and Paediatrics, under consultant supervision in an accredited hospital. That single requirement has become the tightest chokepoint in the entire medical workforce pipeline.

- About **6,000 doctors qualify each year**; the centralised housemanship system has capacity for about **4,000**. Roughly **2,000 graduates are stranded annually**. These are the Council's own figures, presented by the MDCN Registrar to the Senate Committee on Health.
- The stranded graduates are not failing candidates. They have passed every examination and met every academic requirement. They are simply told there is no room.
- Waiting times of **six to twelve months** are now common, and competition for the limited slots has created space for **unofficial payments and other improper fees**. Graduates have publicly reported being asked for sums such as **N400,000** to secure a place, and describe sought-after centres where about 30 slots draw over 300 applicants. These pressures fall hardest on graduates from families least able to pay.

The human cost is a cohort of young, fully qualified doctors kept idle at the exact moment the country needs them most, and at the exact moment they are deciding whether their future is in Nigeria or abroad.

2. Why this matters now more than ever

First, the pipeline is being deliberately widened at the top. As part of the Government's own workforce strategy, admission quotas for medical training have risen by around 38 per cent since 2023, the number of medical schools has grown, and roughly twenty more are in the accreditation pipeline. New intakes are reported to have risen by about 160 per cent between 2023 and the end of 2025. This is the right ambition. But every one of those additional students becomes a graduate who needs a housemanship place. If internship capacity stays frozen at 4,000 while output climbs toward 8,000 and beyond, the number stranded each year does not creep up, it multiplies.

Second, the bottleneck feeds the very emigration the Government is fighting. More than 8,500 Nigerian doctors joined the United Kingdom register between 2021 and 2024 alone, and by some measures around half of Nigerian-trained doctors now practise abroad. A graduate forced to sit idle for a year, hustling for a placement and sometimes asked to pay for it, receives a powerful early signal that the system does not value them. Many act on it. Each departure also writes off a public training investment estimated in the tens of thousands of dollars per doctor. The housemanship bottleneck is not separate from the japa crisis; it is one of its first and most avoidable causes.

Clearing the housemanship bottleneck does not require training a single additional doctor. It requires letting the doctors we have already trained start work. Few reforms in the health sector offer so much return for so little cost.

3. Root-cause diagnosis

The symptoms, bribery, delay, and the friction of centralised posting, all trace back to four distinct and addressable causes. They are set out separately below because each needs its own remedy, and together they account for the whole problem.

Cause 1: Accredited capacity is too small and too narrowly defined

The training pool is dominated by federal institutions. Of the 114 hospitals accredited for housemanship, 47 are teaching hospitals, 21 are Federal Medical Centres, 37 are general and specialist hospitals that are mostly state-owned and military, and just 9 are private. Only 44, all of them federal, sit on the centralised posting portal; the other 70 are left outside it entirely, and between them the 44 federal centres advertise only about 2,783 medical housemanship places in a posting cycle. Two design choices keep the private and state pool small:

- **The accreditation criteria for private hospitals are disproportionately restrictive.** Requirements around infrastructure, resident accommodation, and departmental scope are set at a level that many capable, high-volume private hospitals cannot meet, even where the clinical training they could offer is excellent. The bar screens out good trainers rather than bad ones.
- **The slots allocated to those private hospitals that do qualify are too few.** Even an accredited private hospital is typically granted a small quota, far below the number of house officers it could competently supervise. The pool is both shallow and under-used.

A related rule compounds the problem: accreditation generally expects a single hospital to provide all four core postings under one roof. Many otherwise strong hospitals have real depth in two or three of the four disciplines but not all four, and are therefore excluded entirely.

Cause 2: Graduate demand is rising by policy while capacity stays fixed

The Government is expanding medical school intake as a matter of deliberate strategy. There is currently no mechanism that requires internship capacity to expand in step with graduate output. Capacity planning for training posts is not tied to the number of doctors the system is about to produce. The two numbers are managed by different processes and are drifting apart.

Cause 3: The funding of the intern post is fragile and non-portable

House officers are paid on the first grade of the Consolidated Medical Salary Structure, funded through the federal payroll for federal centres. In practice this funding is unreliable and non-portable:

- Intern salary lines are frequently not captured cleanly in institutional annual budgets, so hospitals decline to take house officers even where the clinical need is obvious.
- Where posts are funded, payment is often delayed; house officers have publicly reported months of unpaid salaries.
- Because the money is attached to specific federal institutions rather than to the graduate, a willing state or private hospital that could train an intern has no funding stream to do so.

Cause 4: Allocation is centralised, opaque, and easy to game

The centralised posting system was introduced to bring order and fairness to placement. In practice, applied to a supply that is far too small, it has rationed scarcity rather than expanded access. Vacancies and quotas are not transparently published in real time, state hospitals sit largely outside the portal, and graduates compete blind for a shrinking set of preferred slots. The transparently contestable pool is narrower still than the headline numbers suggest, because even within the federal centres a large share

of places is reserved for discretionary allocation by hospital managers. Teaching hospitals commonly route only around 40 per cent of their places to the open portal, reserving the rest for the Chief Medical Director, the Provost, and internal use. That discretionary space is precisely where graduates report slots being sold. Opacity plus scarcity is the condition in which unofficial payments flourish. Centralising the allocation of an inadequate supply did not create the shortage, but it concentrated and exposed it, and made rent-seeking easier.

The four causes and their remedies, at a glance

Root cause	Remedy
1. Accredited supply too small and too narrow (private bar too high, slots too few, single-roof rule)	Broaden supply safely: proportionate criteria, rotational consortium accreditation, larger private and state quotas, national quality standard
2. Graduate demand rising by policy against a frozen pool	Plan internship capacity to the graduate pipeline
3. Intern funding fragile and non-portable	Ring-fence a portable house-officer stipend that follows the graduate
4. Allocation centralised, opaque, gameable (bribes, delay)	Transparent, rules-based matching with anti-corruption safeguards

4. Recommendations

The four reforms below map directly onto the four causes. Each can begin within the current budget cycle. Together they are designed to be comprehensive: fix all four and the bottleneck clears; fix only some and the pressure simply moves.

Recommendation 1: Broaden accredited supply, safely (targets Cause 1)

- **Right-size the accreditation criteria.** Move to proportionate, competency- and outcome-based standards focused on what protects trainees: adequate patient volume and case mix, consultant supervision, structured teaching, and record-keeping. Relax requirements that exclude capable hospitals without improving training, and review the private-hospital criteria specifically against this test.
- **Adopt a rotational, consortium model of accreditation.** Where no single hospital offers all four core disciplines to standard, allow two or three hospitals with complementary strengths to be accredited together as a training network, with house officers rotating between them under a lead coordinating institution. Housemanship does not have to happen under one roof.
- **Raise the slot quotas** of accredited private and state hospitals to match their genuine supervisory capacity, and bring all accredited state hospitals onto the portal.
- **Guarantee quality with a national standard.** Introduce a national housemanship curriculum, a structured logbook, and an end-of-internship competency assessment, so a house officer trained in a private or state hospital, or across a consortium, meets the same standard as one trained in a federal teaching hospital. This standard is what makes expansion defensible.

Recommendation 2: Plan internship capacity to the graduate pipeline (targets Cause 2)

- Establish a simple, published, rolling reconciliation between projected annual graduate output and accredited housemanship capacity, reviewed each year by MDCN and the Ministry together.
- Make the approval of new medical schools and intake increases conditional on a credible plan for the additional internship capacity those graduates will need, so the top and bottom of the pipeline expand together.

Recommendation 3: Ring-fence and make house-officer funding portable (targets Cause 3)

- Create a dedicated, ring-fenced federal funding line for house-officer stipends that **follows the graduate to any accredited centre, public or private**, rather than being tied to specific federal institutions. Funding should enable training wherever capacity exists, not confine it to where the payroll happens to sit.
- Guarantee timely payment under the existing salary structure, and remove the budget-capture barrier that currently lets willing hospitals decline interns for want of a line item.

Recommendation 4: Make allocation transparent, rules-based, and clean (targets Cause 4)

- Publish accredited centres, their quotas, and live vacancies in real time on the portal, so every graduate can see what is available.
- Run allocation on transparent, rules-based, automated matching, with a published appeals and grievance channel.

- Pair this with a clear anti-corruption safeguard and reporting route, so any demand for payment to secure a placement can be reported and acted on. Transparency is the cheapest and fastest cure for the bribery the current scarcity invites.

5. A proven direction, and an open door

None of this is speculative. The Nursing and Midwifery Council of Nigeria, facing the same shortage of internship sites for its own graduates, has moved to accredit a wider range of hospitals, including in the private sector, as clinical training sites. The medical profession can follow the same logic.

More importantly, the MDCN has already told the National Assembly that the solution is to include state and privately owned hospitals in the housemanship system. The regulator, the profession, and the private sector are in agreement on the direction. What is needed now is the political sponsorship to turn that agreement into accreditation reform, a funding mechanism, and a transparent portal, on a defined timeline.

6. The ask, and the Federation's offer

The Healthcare Federation of Nigeria respectfully asks the Honourable Minister of State to:

- **Sponsor a review of housemanship accreditation** to introduce proportionate, competency-based criteria and a rotational, consortium model, and to expand private and state hospital participation and quotas.
- **Commission a national housemanship curriculum and end-of-internship competency assessment** to guarantee quality as capacity broadens.
- **Direct the creation of a ring-fenced, portable house-officer funding line** so that funding follows the graduate to any accredited centre.
- **Mandate a transparent, real-time, rules-based allocation portal** with anti-corruption safeguards.
- **Convene a time-bound implementation task team** drawing together the Ministry, MDCN, and the private sector, with quarterly capacity targets reconciled against graduate output.

The Federation offers more than a request. Its members operate a substantial share of Nigeria's accreditable private hospital capacity, and are willing to help design the rotational model, pilot it, and open their institutions as training sites. The Federation can also help mobilise senior supervisory capacity, including from Nigerian medical networks at home and in the diaspora, to meet the consultant-supervision standards that quality training requires. We are ready to sit down with the relevant offices and work through the detail carefully.

Prepared for the Healthcare Federation of Nigeria by Dr Debo Odulana, a member of the Federation who also serves as President of the Doctors Foundation for Care. Submitted for the review and adoption of HFN leadership ahead of transmission.

Annex: figures and sources for verification

The following figures are drawn from public reporting and official statements, and are provided so that HFN leadership can verify each before transmission.

- 6,000 graduates per year; 4,000 system capacity; about 2,000 stranded annually: MDCN Registrar, Dr Fatima Kyari, presentation to the Senate Committee on Health, reported February 2026 (ThisDay, New Dawn, Daily Trust).
- 114 accredited housemanship hospitals (47 teaching, 21 Federal Medical Centres, 37 general and specialist that are mostly state-owned and military, 9 private); only 44, all federal, on the centralised portal, the other 70 outside it; about 2,783 medical and 211 dental places advertised across the 44 portal centres: MDCN Registrar, Dr Fatima Kyari, quoted in Punch and Punch Healthwise reporting. Directly attributable to MDCN.
- Reports of graduates asked for sums such as N400,000 to secure a place, sought-after centres with about 30 slots against 300-plus applicants, and teaching hospitals routing only about 40 per cent of places to the open portal: named-graduate accounts in Punch and Punch Healthwise reporting.
- Core postings of 12 weeks each in Medicine, Surgery, Obstetrics and Gynaecology, and Paediatrics: MDCN housemanship guidance.
- Medical school intake up about 38 per cent since 2023, new schools added, about 20 in the accreditation pipeline; new intakes up about 160 per cent 2023 to end 2025: Federal Ministry of Health statements, 2024 to 2025.
- More than 8,500 Nigerian doctors joined the UK register 2021 to 2024; an estimated 16,000 doctors lost to emigration over five years against roughly 55,000 licensed; about half of Nigerian-trained doctors practise abroad: General Medical Council data, Punch reporting, and related analyses.
- Cost of training a doctor in Nigeria estimated in the tens of thousands of dollars: Mo Ibrahim Foundation analysis.
- National Policy on Health Workforce Migration approved by the Federal Executive Council, August 2024; Nigeria Health Sector Renewal Investment Initiative: Federal Ministry of Health.
- Nursing and Midwifery Council of Nigeria broadening accredited clinical training sites: NMCN and WHO Africa reporting.